

Pathogen of the Week

Weekly briefing for policymakers · WHO · Africa CDC · ECDC · US CDC · UKHSA · PHAC · open-source signals

Week 21 · Mon 18 – Sun 24 May 2026

Produced 2026-05-19

THIS WEEK AT A GLANCE

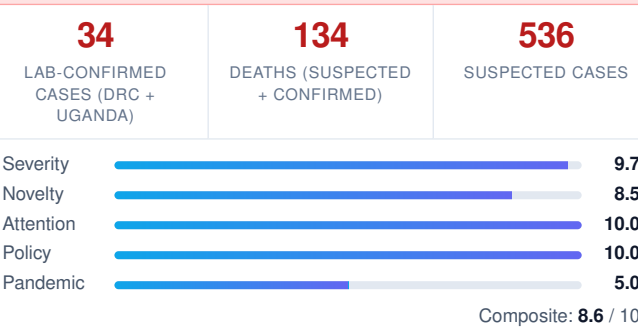
The Ebola Bundibugyo PHEIC has escalated sharply in 72 hours. The US has invoked Title 42 to suspend entry from DRC, Uganda and South Sudan. The MV Hondius docked at Rotterdam on 18 May; contact-tracing now spans 19 countries.

Bottom line: The Bundibugyo ebolavirus outbreak is moving fast. As of 19 May 2026, DRC and Uganda jointly report 34 lab-confirmed, 105 probable and 536 suspected cases, with 134 deaths — up from 10 confirmed and ~80 suspected deaths on 16 May. Twenty-six new confirmed cases and 143 new suspected cases were identified in the last 24–48 hours. An American physician working in Bunia has tested positive and is being medevaced to Germany with six high-risk-contact Americans. On 18 May the CDC invoked Title 42 for the first time since COVID-19, suspending entry of non-US nationals who have been in DRC, Uganda or South Sudan in the previous 21 days. The MV Hondius Andes hantavirus cluster (10 cases, 3 deaths) docked at Rotterdam on 18 May — RIVM reports no symptomatic disembarkees; the contact-tracing window extends ~6 weeks across 19 countries. Mpox clade 1b remains the smoldering background story — ECDC monthly surveillance shows 80 clade I cases reported across 10 EU/EEA countries in March, with sustained transmission.

Ebola Bundibugyo

Bundibugyo ebolavirus (BVD) — DRC Ituri outbreak with cross-border spread to Uganda and first American case

DRC Ituri Province (Bunia, Rwampara, Mongbwalu) · Kampala, Uganda · medevac of US doctor to Germany



WHY IT MATTERS

The PHEIC declared on 16 May 2026 has escalated rapidly. In 72 hours, lab-confirmed cases have more than tripled (10 → 34), probable cases jumped to 105, suspected cases more than doubled (246 → 536), and reported deaths rose from ~80 to 134. The first American case — Dr Peter Stafford, a US physician working in Bunia since 2023 — was confirmed by CDC on 18 May and is being medevaced to Germany alongside six high-risk-contact Americans. The same day the CDC invoked Title 42 for the first time since the COVID-19 emergency, suspending entry of non-US nationals who have been in DRC, Uganda or South Sudan in the previous 21 days; the order is in effect for 30 days. There are still no approved BVD-specific therapeutics or vaccines, but Africa CDC and WHO are convening expert review of investigational monoclonals, remdesivir (which laboratory data suggest may be more active against BVD than against Zaire ebolavirus) and several BVD-specific vaccine candidates for emergency-use clinical trials.

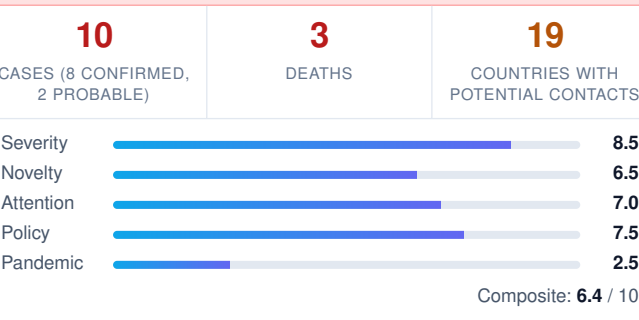
FOR POLICYMAKERS

- Activate cross-border surveillance and exit screening between DRC, Uganda, Rwanda, South Sudan, CAR and Kenya; brief national EOCs on BVD-specific clinical case definitions and rapid PCR panels that distinguish BVD from Zaire ebolavirus.
- Coordinate with WHO R&D Blueprint, BARDA and IFFIm on emergency-use clinical-trial protocols for investigational BVD monoclonal antibodies, remdesivir (IV and oral) and pre-clinical BVD vaccine candidates — existing Zaire-targeted countermeasures cannot be assumed cross-protective.
- Review national equivalents of US Title 42 / Public Health Act entry restrictions carefully: CDC’s own order acknowledges that broad travel bans have limited epidemiologic value and may discourage voluntary self-reporting. Targeted screening, contact registration and conditional release pathways are preferable.

Andes hantavirus

Orthohantavirus andesense (ANDV) — MV Hondius cruise-ship cluster

MV Hondius docked at Rotterdam 18 May 2026 · contacts across 19 countries



WHY IT MATTERS

The Dutch-flagged MV Hondius docked at Rotterdam on 18 May 2026. Dutch port-of-entry authorities, ECDC and WHO coordinated the disembarkation of the remaining 25 crew and 2 medical staff into quarantine. The RIVM reported that none of those coming off the ship were symptomatic. The ship is undergoing disinfection. Sequencing continues to point to limited human-to-human transmission — Andes is the only hantavirus species with documented person-to-person spread — and WHO still assesses global public-health risk as low. The contact-tracing window extends roughly six weeks; passengers and crew who were already repatriated are being followed in their home countries.

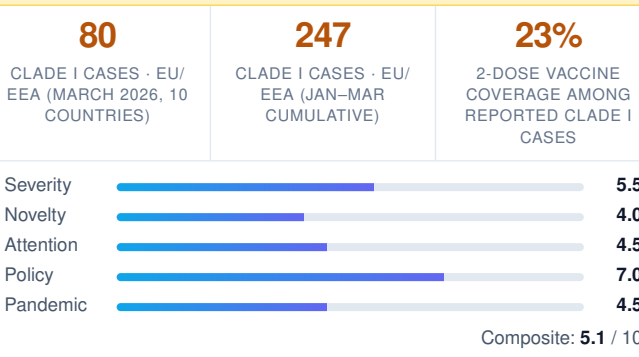
FOR POLICYMAKERS

- Sustain cross-border contact tracing for repatriated passengers and crew across the 19 implicated countries (incubation window up to ~6 weeks; median 14–17 days).
- Support the Dutch RIVM disembarkation and disinfection plan; provide secondary clinical-alert pathways for any contact developing fever or respiratory symptoms.
- Brief cruise lines, port authorities and travel-health authorities on rodent-exclusion, onboard isolation and cabin decontamination protocols — codify for the next event.

Mpox (clade Ib)

Monkeypox virus, clade Ib

DRC, Madagascar, Uganda, Burundi, Kenya · sustained clade I transmission across ≥10 EU/EEA countries



WHY IT MATTERS

ECDC’s new monthly mpox surveillance bulletin shows sustained clade I transmission in Europe, with 80 clade I cases reported across 10 EU/EEA countries in March 2026 (Spain leading with 23 cases), 78 in February and 89 in January — roughly a quarter of the year’s clade I caseload concentrated in EU/EEA Member States. Risk to MSM remains moderate; risk to the general population remains low. Just 23% of clade I cases had completed a two-dose JYNNEOS / MVA-BN course, highlighting persistent vaccination gaps. The DRC outbreak overlap with Ebola Bundibugyo compounds the operational burden on national health authorities. A novel clade Ib/Ilb recombinant remains flagged (WHO DON 2026-DON595).

FOR POLICYMAKERS

- Sustain targeted JYNNEOS / MVA-BN vaccine availability for at-risk groups; address coverage gap — 77% of reported cases were unvaccinated or partially vaccinated.
- Maintain non-stigmatizing messaging through community organizations; avoid travel restrictions for clade Ib.
- Fund continued genomic surveillance — clade I/II recombinants are emerging; ECDC has now committed to monthly public reporting.

